

中國醫藥大學附設醫院(CMUH)
病理檢查報告
Pathology Report

2616434 (中山醫學大學附設醫)

CM-Y000-721517

病理號:P202604711 來源:中山醫學大 台中市南區建國北路1段110號

申請:(1150326) 檢查:1150326 報告:1150417

姓名:增豐 出生日: 性別: 住院號:OUTPATIENT

報告醫院地址:台中市北區育德路2號 電話:(04)22052121 分機:12015/12676

Hospital add.:No.2,Yude Raod,Taichung City,40447 Taiwan (+886)4-22052121 ext.12015/12676

[採檢資訊]

顏色: 數量: 種類:
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25004C X 1 外科病理檢查

25010B X 3 組織化學染色第一類Histochemical stain group I

25012B X 2 免疫組織化學染色 Immunohistochemical stains, each ant

25013B X 7 螢光切片檢查第一類 Immunofluorescence group I, each a

[病理報告]

Kidney, graft, 5- years post transplantation, echo-guided needle core biopsy,

a. Acute T cell mediated graft rejection, Banff Grade IIA, acute tubular injury and mildly chronic changes of the parenchyma.

b. Banff score: g0, il, til, i-IFTA1, tl, t-IFTA0, v1, ptc2, cg0, mm0, ptcml0, cil, ctl, cvl, ah0, aah0, c4d0, pv10

See Comment.

Comment

The renal biopsy shows features of active graft rejection, Banff Grade IIA, characterized by the presence of intimal arteritis (v1). While the overall interstitial inflammation is relatively mild (il), the vascular involvement is the defining feature for Grade IIA. Although C4d stain is negative, the possibility of combined effect of antibody mediated rejection cannot be excluded. Correlate with clinical findings, DSA status and follow up are needed.

[描述]

Hospital: 中山醫大附設醫院

The submitted specimen consists of 1 tissue core, measuring 1.5x0.1x0.1 cm, in fresh state, submitted for immunofluorescent examination; and 1 tissue core, 0.8x0.1x0.1 cm, fixed in formalin, submitted for light

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microscopic examination.

Grossly, they are whitish gray and soft.

All for sections and prepared for routine serial H&E staining (x 4), histochemical study (PAS x 1, Jones silver x 1, and Masson's trichrome x 1), immunohistochemistry stains (SV40 and C4d), and DIF study (direct IF x 7, HE x 1, and PAS x 1). Jar 0.

Specimen adequacy: Adequate, Cortex and soft tissue

Light microscopy:

1. Glomerulosclerosis
 - 1.1 Number of glomeruli: (6)
 - 1.2 Number globally sclerotic: (0)
 - 1.3 Segmental sclerosis: (0)
2. Number of arteries with internal elastic lamina: (3)
3. Glomerulitis (g), score: 0 (none)
4. % inflammation with non-scarred cortex areas, exclude subcapsular cortex (i), score: 1
 - a. % Total cortical inflammation, including scarred and non-scarred cortex (ti), score: 1
 - b. % inflammation in scarred cortex, exclude subcapsular cortex (i-IFTA), score: 1
 - c. Plasma cells >15/hpf over 5 fields: Absent
5. Tubulitis in cortical tubules within non-scarred cortex (t), score: 1
 - a. Tubulitis in tubules within scarred cortex, exclude severely atrophic tubules (t-IFTA), score: 0
6. Intimal arteritis in arteries with minimum 2 smooth muscle layers (v), score: 1
7. Peritubular capillaritis (ptc), score: 2
8. Chronic glomerulopathy (transplant glomerulopathy, cg), score: 0
 - a. Mesangial matrix increase (mm), score: 0
 - b. PAS and MST glomerular basement membrane changes: No
9. Peritubular capillary basement membrane multilayering (EM) (ptcml), score: 0
10. Interstitial fibrosis in cortex exclude subcapsular cortex (ci), score: 1
11. Tubular atrophy in cortex exclude subcapsular cortex (ct), score: 1
12. Arterial intimal fibrosis (fibrointimal thickening, cv), score: 1

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13. Arteriolar hyalin (ah), score: score: 0

a. Nodular form (aah): Not identified

14. Other findings: Focal acute tubular injury

15. C4d: Negative, score: 0

16. BKV: Negative; Intrarenal polyomavirus load level (pvl), score: 0

17. Banff Diagnostic categories: (2019)

Category 4: T-cell mediated rejection (TCMR), IIA

Immunofluorescence microscopy (DIF):

* Number of glomeruli in frozen section: (7); global: (0); segmental: (0). Artery (1)

* Essential markers: IgG, IgA, IgM, C3, Clq, Kappa, Lambda light chains
Intensity: 0-3+/3+

* No specific immune deposition is found in glomeruli, including IgG, IgA, IgM, C3, Clq, Kappa and Lambda light chains.

* No specific immune deposition is found in tubulointerstitium nor blood vessels, including IgG, IgA, IgM, C3, Clq, kappa and lambda light chains

Transmission electron microscopy (E2026082):

* Number of glomeruli included in semi-thin section: (6), none sclerosis

* Number of glomeruli examined ultrastructurally: (3)

* Glomerulus:

Normal cellularity and patent capillary

Glomerular basement membrane (GBM): normal

Measurement: mean 304 nm

Contour: Regular

Texture: Negative for splitting, disruption, spikes

Capillary loops: Negative for subendothelial neo-dense glomerular basement membrane, Negative for endothelial cell enlargement, Negative for loss fenestration or subendothelial electron-lucent widening

Electron dense immune type deposits: Absent

Mesangial interposition: Not identified

Foot process effacement: Focal, less than 30 %

* Number of PTC examined (11); the number of BM layers in the most affected PTC (3);

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Banff 2013 threshold for chronic ABMR (ptcml1; 1 PTC with ≥ 7 layers +2 PTC with ≥ 5 layers) is met: No

* Tubular cells: Tubular epithelial cells exhibit prominent apical blebbing into the lumen and loss of the brush border. Negative for significant interstitial inflammation.

Clinical findings:

66 years old Male

Medical History: End-stage renal disease (ESRD), s/p Hemodialysis since 2021/05. S/p Kidney transplant in 2021/08 (Mainland China).

Post-transplant diabetes mellitus (PTDM), Hypertension.

Medication: Cyclosporine, MMF (Mycophenolate Mofetil), and Prednisolone.

Present Illness: Post-transplant regular follow-up at Sun Yat-sen

Memorial Hospital (廣州中山). Elevated serum creatinine noted during the past 2 months (Baseline ~ 1.3 mg/dL up to 1.6 mg/dL then 1.9 mg/dL).

Current lab shows Cr 1.73 mg/dL (eGFR 42.9 mL/min/1.73m²) and BUN 55 mg/dL. Absence of proteinuria or hematuria in urine analysis.

Lab Data:

Urine Analysis: No proteinuria, no hematuria.

Urine Protein/Cr Ratio (UPCR): 64.0 mg/g.

BUN 55 mg/dL, Cr 1.73 mg/dL, eGFR 42.9 mL/min/1.73m², Alb 4.3 g/dL, Uric

Acid 4.5 mg/dL, Cholesterol 152 mg/dL, Triglyceride 553 mg/dL,

Hgb 11.2 g/dL, PLT 109,000 /uL.

Cause of Biopsy: Acute Kidney Injury in a renal allograft, suspicion of acute rejection

Attending Pathologist: Mei-Chin Wen

Signout by: Dr. 文美卿 (病解專醫字217)



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CM-Y000-721518

病理號:E2026082 來源:中山醫學大 台中市南區建國北路1段110號

申請:(1150326) 檢查:1150326 報告:1150417

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25014B X 1 電子顯微鏡切片檢查Electron microscopy(委外)

[病理報告]

Kidney, graft, biopsy (electron microscopy), Acute tubular injury.
Negative for electron dense deposit. Negative for transplant
glomerulopathy. Negative for peritubular capillaries basement membrane
multilamination. (See Note)

Note:

Please refer to P202604711 for the detailed pathology findings and
report.

[描述]

Hospital: 中山醫大附設醫院

Reference: P202604711

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Mesangial interposition: Not identified

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